

Tobacco Cessation: risk questionnaire

Tobacco cessation — risk & readiness questionnaire

Two-part self-assessment for tobacco users:

- **Part A** — nicotine dependence (adapted from the Fagerström Test for Nicotine Dependence, the de-facto standard tool, simplified for patient self-administration; mirrors the screening items in Annex 1 of the WHO *Be He@lthy Be Mobile — mTobaccoCessation handbook*).
- **Part B** — readiness to quit (adapted from the Transtheoretical Model “stages of change” framework).

This is a triage tool, not a diagnosis. The combined score guides what kind of support is likely to help most, not whether you “can” quit — people at every dependence level have successfully quit.

Part A — Nicotine dependence

A1. When do you smoke your first cigarette of the day?

- (3) Within 5 minutes of waking
- (2) 6 - 30 minutes after waking
- (1) 31 - 60 minutes after waking
- (0) After 60 minutes

A2. Do you find it difficult to refrain from smoking in places where it is forbidden (a hospital, a cinema, an office, an aeroplane)?

- (1) Yes
- (0) No

A3. Which cigarette of the day would you most hate to give up?

- (1) The first one in the morning
- (0) Any other one

A4. How many cigarettes per day do you usually smoke? (For other tobacco products, count comparable use — e.g. one cigar $\approx$ 2-3 cigarettes; one pipe session $\approx$ 1 cigarette; one shisha session $\approx$ many cigarettes — see special-cases below.)

- (0) 10 or fewer
- (1) 11 - 20
- (2) 21 - 30
- (3) 31 or more

A5. Do you smoke more frequently during the first hours after waking than during the rest of the day?

- (1) Yes
- (0) No

A6. Do you smoke even when you are so ill that you have spent most of the day in bed?

- (1) Yes
- (0) No

Dependence score (Part A total)

Total	Dependence level	What this typically means for quitting
0 - 2	Low	Many people at this level can succeed with behavioural support and a clear plan. Pharmacotherapy is optional.
3 - 4	Moderate	Behavioural support plus pharmacotherapy (NRT, bupropion or varenicline depending on what's available and your health worker's advice) noticeably increases success.
5 - 7	High	Withdrawal will be intense in the first 1-2 weeks. Pharmacotherapy is strongly recommended. Plan for daily support during the first month.
8 - 10	Very high	This is the addiction working hard. Quitting is absolutely possible, but it usually needs combination pharmacotherapy plus structured behavioural support (counselling, programme, regular check-ins). Talk to a health worker before Quit Day.

Part B — Readiness to quit

B1. Which of these is closest to how you feel right now?

- (0) I have no intention of quitting in the next 6 months. (*Pre-contemplation*)
- (1) I'm thinking about quitting in the next 6 months but not in the next month. (*Contemplation*)
- (2) I'd like to quit in the next month and I've already started thinking about how. (*Preparation*)
- (3) I quit within the last 6 months. (*Action*)
- (4) I've been smoke-free for more than 6 months. (*Maintenance*)

B2. How confident are you that you could stay smoke-free for an entire day, if you tried right now?

- (0) Not at all confident
- (1) A little
- (2) Moderately
- (3) Very

B3. Do you have at least one person in your life who would actively support you through a quit attempt (a partner, a family member, a friend, a colleague, a community group)?

- (2) Yes, definitely
- (1) Maybe — I haven't asked
- (0) No

B4. Have you tried to quit before?

- (1) Yes, at least once
- (0) No

(If yes:) **B4a.** What was the longest you stayed smoke-free?

- (2) More than 6 months
- (1) Between 24 hours and 6 months
- (0) Less than 24 hours, or only “cut down”

Readiness score (Part B total)

Total	Stage	Recommended next step
0 - 2	Pre-contemplation / very low readiness	Don't pressure yourself. The fact you're reading this is already a step. Learn one new thing about how your body is affected, and revisit this in a few months.
3 - 5	Contemplation	Pick one specific reason to quit (children, breath, money, a specific health concern). Write it down. Notice when you smoke and what triggers it for a week. No need to quit yet — just observe.
6 - 8	Preparation	You're ready. Pick a Quit Day in the next 1-2 weeks. Decide on whether you'll use pharmacotherapy (Part A score above moderate? Strongly recommended). Tell at least one supporter.
9 - 12	Action / Maintenance	Keep going. Recognise the milestones (day 1, week 1, month 1, month 3, month 6, year 1). If a slip happens, treat it as data, not failure.

Combining the two scores

Part A (dependence)	Part B (readiness)	Suggested path
Low / Moderate	Pre-contemplation / Contemplation	Motivational support only. No quit plan yet.
Low / Moderate	Preparation	Behavioural support, Quit Day planning, optional NRT.
Low / Moderate	Action	Daily check-ins for first 2 weeks, weekly to month 3.
High / Very High	Pre-contemplation / Contemplation	Motivational support; mention that medication exists and works.
High / Very High	Preparation	Pharmacotherapy + behavioural support together. Strongly recommend a clinic visit before Quit Day.
High / Very High	Action	Daily check-ins. Watch for severe withdrawal — recommend NRT if not yet using.

Special-case adjustments

- **Hookah / shisha only:** Treat the dependence score with caution. Many shisha users score “low” on the cigarette-oriented questions but inhale large doses during sessions. If they have hookah $\geq$ once/week, treat the addiction as moderate at minimum.
 - **Smokeless tobacco (chewing, snus, dip):** Adapt questions A1, A3, A5 to “use” rather than “smoke.” Scoring is similar. Add the question: do you ever wake up at night to chew/dip? (yes = high dependence).
 - **Pregnant or breastfeeding:** Regardless of dependence score, the recommendation is to quit immediately, with a clinician-supervised plan. Risk to the baby of continued smoking outweighs the inconvenience of dependence-level withdrawal in nearly all cases.
 - **Recent cardiovascular event (heart attack, stroke) within last 2 weeks:** Quitting is urgent. Pharmacotherapy choice must be decided with the treating clinician.
 - **History of depression or bipolar disorder:** Flag for clinician involvement. Mood drops in the first weeks post-quit are more common in this group and may need adjustment to existing treatment.
-

What this questionnaire does not do

- It does not predict your individual chance of success. People at every dependence level have quit; people at every readiness level have, at some point, become ready.
- It does not replace conversation with a health worker. If your combined picture is “high dependence + high readiness,” the most useful single next step is a clinic visit to discuss pharmacotherapy before your Quit Day.
- It does not assess withdrawal severity directly — that becomes visible only after you stop. If withdrawal becomes overwhelming, that is also a reason to seek clinical support, not to give up.